

CRIMINAL ENTERPRISE AND ORGAN TRAFFICKING: HOW FINANCIAL REGULATION AND ANTI-MONEY LAUNDERING LAWS AFFECT HUMAN ORGAN MARKETS

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Introduction

Organ trafficking is largely a consequence of the worldwide scarcity of viable organs for ethical transplant operations. Over 150,000 ethical transplants occur annually, which only meets 10% of the global demand for organs. The organ trade market has made between 840 million and 1.7 billion USD annually.⁸²² Organ trafficking requires tactical planning as it involves medical specialists, healthcare facilities, and complex global networks. Criminal groups have typically targeted unemployed individuals or those from poor, uneducated, and vulnerable backgrounds. Through organized networks, recipients of donors are typically from “demand countries” such as Europe and North America. Donors often come from developing countries, and traffickers have typically targeted armed conflicts and refugees. For example, since the beginning of the Syrian conflict, 20,000 organ sales have occurred.⁸²³ Shell companies and underground companies have played a key role in the merging of criminal networks and their involvement with organ trafficking. Shell companies are used as they are able to launder illicit money and conceal the identities of those involved. These companies have no independent operations or significant assets, which are used to mix “dirty” money with legitimate revenue, making it difficult for financial institutions to detect criminal

⁸²² United Nations Office on Drugs and Crime, “Explainer: Understanding Human Trafficking for Organ Removal,” United Nations : Office on Drugs and Crime, June 2024, https://www.unodc.org/unodc/frontpage/2024/June/explainer_-_understanding-human-trafficking-for-organ-removal.html.

⁸²³ Thomas Martial, “Harvesting Vulnerability: The Challenges of Organ Trafficking in Armed Conflict,” International Review of the Red Cross, June 27, 2023, <https://international-review.icrc.org/articles/harvesting-vulnerability-the-challenges-of-organ-trafficking-in-armed-conflict-923>.

involvement. Transactions are made through complex international structures, as these shell companies use payment processes for transplant tourism, foreign procedures intended for illegal transplants. The usage of fake travel agencies or fraudulent medical tourism websites pretending to offer “legal” transplants creates an appearance of legitimacy. Organ trafficking persists as criminal groups exploit weak financial oversight, cash-based transactions, cross-border networks, and corrupt medical infrastructure. Although organ trafficking is primarily viewed as a human rights and medical ethics issue, the organ trafficking market can succeed as it is financially lucrative; therefore, applying financial regulations and Anti-Money Laundering frameworks can substantially disrupt these networks.

Background

Organ trading is the practice in which organs are treated as a commodity, meaning they are bought or sold for material gain or comparable advantage to the donor. The sale of organs is widely condemned and illegal in most countries based on the ethical principle that organs should be a gift rather than exchanged through market transactions. Framing organs as gifts is intended to preserve human dignity, prevent the exploitation of economically vulnerable populations, and avoid creating a system in which access to life-saving transplants is determined by wealth rather than medical need. The World Health Organization defines organ trafficking as the exploitation of an individual using coercion, deception, or abuse of position.⁸²⁴ Transplant tourism is the act in which patients travel across borders with the intent to obtain a transplant. To be considered “transplant tourism,” it must involve either organ trafficking or organ transplantation and undermine the ability of a host country to provide

⁸²⁴ “Organ Trafficking and Transplant Tourism,” American Society of Transplantation, 2018, <https://www.myast.org/organ-trafficking-and-transplant-tourism>.

transplant services for its own citizens. The UNODC's definition of organ trafficking is primarily dependent on the Palermo Protocol, which focuses on the means, such as force or coercion, used to control a person for organ removal. The 2015 Convention against Trafficking in Human Organs defines organ trafficking as a criminal activity entailing the removal of organs for financial gain without informed consent, as well as the related transportation, brokering, and use of such organs.⁸²⁵ Transnational criminal organizations are the backbone behind the illegal organ trade as they coordinate the entire process and exploit vulnerable populations such as refugees or undocumented migrants, luring them in with false promises of money or potential job opportunities. These organizations also rely on working with individuals in the legitimate medical sector, as organ trafficking requires specialized medical knowledge. These organizations also handle the forging of illegal documents to disguise the illegal nature of the transplant and arrange travel accommodations necessary. Transnational crime organizations ensure that a majority of profits end up with high-level members and brokers, leaving donors with little to no money.

Stages of Organ Trafficking

The typical supply chain for organ trafficking is a complex one, but can be broken down into the stages of recruitment, logistics and matching, removal and transplantation, financial transactions, and post-operative neglect. During the recruitment process, brokers and local recruiters recruit and identify potential victims from vulnerable communities using deception, coercion, and abuses of power to obtain “consent” for organ removal. Victims are often lured with false promises of compensation, employment, or medical care. Migrants,

⁸²⁵ “Trafficking in Human Organs - FAQ” (Council of Europe, May 28, 2025).

refugees, and individuals facing extreme poverty are particularly susceptible to recruitment efforts.

Following the selection of a potential donor, organ traffickers coordinate logistics to match the organ with a recipient. This stage could involve falsifying medical records, forging documentation to imply familial relationships, and arranging travel to jurisdictions with weaker regulatory oversight. Medical testing is done in secret with compliant medical facilities to minimize risks.

The surgical removal and transplantation of organs typically occur in private clinics or hospitals where procedures are performed in substandard conditions. Procedures often prioritize speed over donor welfare. In many cases, donors are not fully informed of the risks associated with the medical risks involved or the long-term consequences of organ removal.

Financial transactions are deliberately perplexing, as substantial payments from recipients are funneled through shell companies, brokers, or informal money transfer systems. Donors only receive a small portion of the payment, while intermediaries gain the majority. These transactions are tactfully created to avoid financial monitoring and anti-money laundering regulations.

In the final stage of the process, donors are typically abandoned without adequate medical follow-ups, compensation, or legal resources. Many victims experience long-term health complications, loss of employment, reinforcing cycles of poverty and vulnerabilities that traffickers can exploit.

International Regulation

Internationally, much of the legislation regarding organ trafficking has significant limitations. The United Nations Palermo Convention of 2000 addresses trafficking in persons for organ removal, which offers tools for cross-border cooperation, extradition, assistance for victims, and information sharing. The limitations of this, however, are that organ trafficking is not always explicitly covered unless correlated with broader human trafficking offenses. Likewise, the Protocol requires elements of coercion, abduction, fraud, or abuses of power to establish an offense. This definition fails to cover cases in which impoverished individuals “consent” to selling organs for financial gain, and the vulnerable state of donor victims is often difficult to prove in court. The protocol also does not explicitly criminalize the purchase or sale of organs or the act of transplant tourism. Furthermore, enforcement varies across states, especially where corruption is prevalent. As the Protocol relies heavily on individual nation states to implement and enforce domestically, many countries have insufficient national laws, or laws in which it is difficult to prosecute crimes committed abroad. The Declaration of Istanbul defines both “organ trafficking” and “transplant tourism” and encourages states to criminalize organ sales and create ethical transplant systems.⁸²⁶ However, the Declaration is largely a professional ethical framework and not a legal instrument that has sanctioning power or monitoring. The Council of Europe Convention against Trafficking in Human Organs and its Additional Protocol on Organ Transplantation are legally binding European agreements establishing frameworks to combat organ trafficking by criminalizing illegal removal, protecting victims and international cooperation, and setting standards against the buying and selling of organs. Domestically, the 1984 United States National Organ Transplant Act (NOTA) prohibits the buying or selling of human organs for transplantation while also

⁸²⁶ Clin Nephrol, “The Declaration of Istanbul on Organ Trafficking and Transplant Tourism,” *Clinical Journal of the American Society of Nephrology* 3, no. 5 (August 13, 2008): 1227–31, <https://doi.org/10.2215/cjn.03320708>.

establishing the Organ Procurement and Transplantation Network (OPTN) to manage the national system for organ sharing, matching, and allocation, which is overseen by a private non-profit. The primary criticism of NOTA has been the ban on “valuable consideration,” as it prevents compensation for organs, which significantly limits the supply of organs despite the growing demand. Moreover, the Act has been criticized for the sequential allocation model, which offers organs to individuals in order, a system that is considered to be ill-suited for modern, geographically vast networks, which fail to match organs with recipients who will receive the greatest benefit. While NOTA allows for the reimbursement for donor travel, housing, and lost wages, it forbids any additional forms of compensation. This restriction limits the scope for innovative incentive-based solutions that could ethically encourage donation. By confining compensation to recovery costs alone, NOTA prevents policymakers and medical institutions from experimenting with regulated, non-coercive incentives such as long-term health benefits, tax credits, and education grants. While NOTA prioritizes the prevention of exploitation, this legislation also constrains the efforts to address the shortage of viable transplantable organs.⁸²⁷ Similarly, NOTA is written for domestic donation markets, not international criminal networks. NOTA also does not address organ trafficking as a transnational crime or provide any financial regulatory provisions for tracking payments or supply-chain financial flows.

Domestic Regulation

One of the financial and Anti-Money laundering regulatory measures that are relevant to organ trafficking is the Bank Secrecy Act (BSA) and FinCEN regulations. Financial

⁸²⁷ “Organ Procurement and Transplantation: Administration, Oversight, and Policy Issues,” 2025, https://www.congress.gov/crs_external_products/R/PDF/R48426/R48426.2.pdf.

institutions are required to report suspicious activities (SARs) for potentially illegal transactions, and could be considered useful in detecting unusual medical travel payments or clustered transfers.⁸²⁸ While financial institutions must report transactions greater than \$5,000, with no clear purpose, the main limit is that organ typologies are not clearly detailed, meaning that banks typically miss the red flags involved with organ trafficking related situations. Payments for transplant tourism meet the description for having “no apparent lawful purpose.” FinCEN regulations typically focus on general money laundering, making it more difficult for banks to connect the dots, leading to underreporting by financial institutions, as there is difficulty in proving intent. Furthermore, the Financial Action Task Force (FATF) sets the global standards for Anti-Money Laundering (AML) obligations to detect and disrupt illicit financial flows, including for human/migrant smuggling. There are 40 recommendations in which countries form the global benchmark, requiring countries to establish AML systems such as customer due diligence, record-keeping, and suspicious transactions reporting⁸²⁹. One of the regulatory gaps that has occurred with the Financial Action Task Force’s recommendations is that organ trafficking is not always considered to be a predicate offense (a crime that generates illicit funds), which has created loopholes. Criminals have exploited these gaps by using medical tourism agencies, NGOs, and cryptocurrencies to move money for these transactions.

Limitations of Current Frameworks

⁸²⁸ “Financial Crimes Enforcement Network,” n.d., https://www.fincen.gov/system/files/shared/sarnarrcompletguidfinal_112003.pdf.

⁸²⁹ “What Is FATF? Role in Global AML Standards - Glossary,” Lseg.com, 2025, <https://www.lseg.com/en/risk-intelligence/glossary/aml/financial-action-task-force>.

Despite the existence of organ-specific criminal statutes and broad financial oversight measures, there lie several structural limits undermining effective enforcement against organ trafficking. These limitations have stemmed from jurisdictional inconsistencies, lack of transparency with financial flows, and gaps in law and enforcement capacity. Countries vary dramatically in their criminalization thresholds, as some countries have punishments as minute as compensation bans, whereas others have ruled it as medical malpractice. Especially in countries with corruption and weak enforcement capabilities, weak enforcement enables illicit transplant hubs. This fragmentation between rules has created safe jurisdictions where traffickers can move donors, patients, and financial flows with minimal legal exposure. As organ trafficking is a highly specialized process, the medical sector's opacity alongside professional gatekeeping has left organ trafficking cases highly underreported. The lack of mandatory international reporting of transplant tourism cases has led to a lack of awareness about organ trafficking. The rules regarding protected medical privacy have also limited financial regulators' ability to link suspicious payments with medical procedures. Many transplant facilities are also present in high-risk regions lacking standardized oversight. These gaps have allowed criminal networks to route payments with legitimate medical providers without getting caught. AML systems are also not designed with the intention of organ markets. Banks and regulators are rarely given training and outlines on flagging transplant-related payments. Financial monitoring predominantly focuses on terrorism, drugs, and fraud, not illegal surgeries. The lack of suspicious activity indicators has also limited reporting on the issue. Additionally, with the absence of information sharing between hospitals, NGOs, border agencies, and financial institutions, organ traffickers can disguise payments with general medical tourism channels and cryptocurrency transfers.

Conclusion

Human organ trafficking can operate at the intersection of criminal networks, transnational markets, and regulatory blind spots. Although international conventions and domestic criminal statutes have prohibited the trade of these organs, enforcement is inconsistent, and criminal networks can exploit financial opacity. As organ trafficking is a profit-driven market, the financial infrastructure enables it. Strengthening financial regulation and Anti-Money Laundering enforcement through stronger reporting obligations and cross-border data sharing has the potential to end the organ trafficking market. By framing organ trafficking as a financial crime, regulators can use existing AML systems to detect suspicious payments and follow the structure of organ trafficking networks. Addressing organ trafficking requires strengthened financial surveillance and coordinated international governance. Further embedding AML mechanisms can diminish the intermediaries and financial anonymity that allow these markets to survive. Shrinking the financial space in which organ traffickers operate brings the world closer to ending the treatment of bodies as commodities.